

Period Delay (Norethisterone)

Patient Group Direction, version 005, issued 9 September 2026. Norethisterone 5mg tablets, women aged 16 years and over.

The three numbers that changed

- **MAXIMUM 14 DAYS** of treatment. 14 days is the figure that now applies throughout.
- **MAXIMUM 42 TABLETS** per supply. At 5mg three times daily that is exactly 14 days.
- **VTE risk factors** are now **EXCLUSIONS**, not cautions. See the gate below.

Why the venous thromboembolism gate is real

Norethisterone at 5mg three times daily is a high dose. A clinically significant proportion is metabolised to ethinylestradiol, so the venous thromboembolism risk at this dose sits closer to that of a combined oral contraceptive than to a progestogen-only pill. It is not a low-risk hormone taken for a fortnight.

- **That gates nothing and is not auditable.**
- **Long-haul travel is both the commonest reason a woman asks for this and one of the risk factors. That tension is resolved in favour of safety: a journey of 4 hours or more during the course, or within 2 weeks of finishing it, is an exclusion.**
- **Where a woman is excluded only because of travel, tell her what she can do instead. See Appendix 2.**

Patients aged 16 and 17

- **This PGD covers 16 and 17 year olds. Every supply to a patient under 18 requires a recorded competence assessment and a recorded consideration of safeguarding.**
- **Ask why the delay is wanted and who has suggested it. Where the request appears to come from someone other than the patient, or the reason given is inconsistent, do not supply. Refer.**
- **Consider whether the request may indicate coercion, exploitation, or an attempt to conceal something. A request to delay a period is not neutral in a young person.**
- **Record the assessment, not just its conclusion.**

Norethisterone 5mg for the short term delay of menstruation

Patient Group Direction for the supply of norethisterone 5mg tablets to delay menstruation in women aged 16 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be familiar with the current SPC for norethisterone 5mg tablets, the BNF and NICE CKS.
- All users must understand that norethisterone at this dose carries a venous thromboembolism risk closer to a combined oral contraceptive than to a progestogen-only pill, and must be able to explain that to a patient.
- All users must be competent to assess competence and consent in a 16 or 17 year old, and to recognise and act on safeguarding concerns including coercion and exploitation.
- All users must know the local safeguarding referral route and be able to use it without delay.
- All users must previously have supplied medicines under a PGD.
- All users must be up to date with CPD and appraisal, and with MHRA safety alerts relating to this medicine.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Short term delay of menstruation in a woman aged 16 years and over with a regular menstrual cycle, who is not using hormonal contraception, and who wishes to postpone her period for a personal, social, religious or travel-related reason. This is a lifestyle supply, not a treatment for a medical condition.
Inclusion criteria	• Female, aged 16 years or over. • A regular, predictable menstrual cycle, so that the start date can be calculated. • The expected date of the next period is known and is more than 3 days away. • Not using hormonal contraception. Women on a monophasic combined pill should run packs back to back instead, which is safer and free. See Appendix 2. • Pregnancy excluded. See the pregnancy row below. • Valid informed consent obtained. Where the patient is 16 or 17, a competence and safeguarding assessment recorded. • No exclusion criterion present.
Exclusion criteria: venous thromboembolism gate, mirroring UKMEC 2025	EXCLUDE and refer where ANY of the following is present. These mirror UKMEC 2025 category 3 and 4 for combined hormonal contraception, plus the SPC contraindications. • Personal history of venous thromboembolism, deep vein thrombosis, pulmonary embolism, stroke, transient ischaemic attack, myocardial infarction or any arterial disease. • Known thrombophilia, or a first degree relative with venous thromboembolism under the age of 45. • AGED 35 OR OVER AND CURRENTLY SMOKES, any number. UKMEC 3 below 15 a day, UKMEC 4 at 15 or more. Under 35 a smoker is UKMEC 2 and is handled as a risk factor below, not an exclusion. • AGED 35 OR OVER AND STOPPED SMOKING LESS THAN A YEAR AGO. UKMEC 3. Ask this separately: a question that only asks "do you smoke" misclassifies a recent quitter. • BMI 35 kg/m² OR ABOVE. UKMEC 3. Measure or ask for height and weight; do not estimate by looking. BMI 30 to 34.9 is UKMEC 2 and is a risk factor below, not an exclusion. • Surgery under general anaesthetic within the last 6 weeks, or planned during or within 2 weeks of the course. • Any current or expected period of immobility, including a leg in plaster or being bed-bound.

	• Active cancer, or cancer treated within the last 12 months.
Exclusion criteria: all other	EXCLUDE, refer, where any of the following applies. • Known or suspected pregnancy, or pregnancy cannot be excluded. • Breastfeeding. • Aged under 16. Male. • Irregular or unpredictable menstrual cycle. • Undiagnosed vaginal bleeding, bleeding between periods, or bleeding after sex. Refer; this needs a diagnosis, not a delay. • Liver dysfunction, active liver disease, a history of jaundice in pregnancy, or a liver tumour. • Any hormone sensitive cancer, including breast cancer, current or past. • Migraine with aura, or any migraine with focal neurological symptoms, current or past. • Acute porphyria, or severe pruritus in a previous pregnancy. • Hypersensitivity to norethisterone or to any excipient in the product supplied. • Taking an enzyme inducing medicine: rifampicin, rifabutin, carbamazepine, phenytoin, phenobarbital, primidone, topiramate, efavirenz, ritonavir or St John's wort. These reduce norethisterone efficacy and the delay is likely to fail. • Taking ciclosporin, or lamotrigine as monotherapy. • Diabetes with vascular complications. • Uncontrolled hypertension, or a blood pressure of 140/90 or above measured today. • Aged 16 or 17 and the competence or safeguarding assessment is not satisfied, or the pharmacist has any concern about coercion. • Already supplied norethisterone for period delay twice in the last 6 months. Refer; repeated requests need review, and total treatment must not exceed 30 days in any 6 month period. • Valid informed consent not obtained.
Excluding pregnancy	Do not supply until pregnancy has been excluded. Ask, in these terms: • "When did your last period start, and was it normal for you?" • "Have you had sex without contraception, or had a contraceptive failure, since that period?" Where the last period was normal, on time, and there has been no unprotected sex since, pregnancy can be excluded on history. Otherwise, do not supply until a pregnancy test taken no earlier than 21 days after the last unprotected sex is negative. Record the date and result. Tell every patient that if her period does not arrive within a few days of finishing the course, she must do a pregnancy test.
Cautions	Norethisterone is NOT a contraceptive. A woman relying on it will become pregnant. Advise condoms or another method throughout and afterwards. May affect blood glucose. Advise a woman with diabetes to monitor more closely during the course. May cause mood changes, breast tenderness, nausea, headache and breakthrough bleeding. May cause fluid retention. Take care where a condition could be aggravated by it, including epilepsy, migraine, asthma, and cardiac or renal impairment. Stop the medicine and seek medical advice at once for: jaundice, a significant rise in blood pressure, a new migraine-type headache, sudden visual or hearing disturbance, or any possible clot symptom.

	DEPRESSION AND MOOD. UKMEC 2025 REMOVED depression as a categorised condition and replaced it with a statement: there is not consistent evidence that hormonal contraceptives worsen or improve anxiety or mood disorders in those with pre-existing conditions. A history of depression is therefore NOT an exclusion from this PGD. What is required instead is individualised counselling: tell every woman that mood change is a recognised effect, ask her to monitor her mood, and tell her to seek follow up if it deteriorates. Record that you did. Where a woman has a history of depression, the Primolut N SPC directs that she be carefully observed and the medicine discontinued if depression recurs to a serious degree. Say that to her in plain terms: if your mood drops noticeably, stop taking it and speak to us or your GP. EXCLUDE and refer only where there is current severe depression, active suicidal ideation, or any safeguarding concern. That is a clinical judgement, not a tick box, and the reason must be recorded.
Actions if the patient is excluded or declines	Explain why supply is not appropriate, and give the alternatives in Appendix 2 rather than leaving her with nothing. Where the exclusion is travel-related, say so plainly and explain the reason. Where a safeguarding concern exists, follow the local safeguarding route the same day and record what was done. Document the advice given and the decision reached. Inform or refer to the GP as appropriate.

The medicine

Name, form and strength	Norethisterone 5mg tablets.
Legal category	POM.
Dose and frequency	One 5mg tablet three times daily, starting 3 days before the expected onset of menstruation. All three doses are taken every day of the course.
Quantity to be supplied	MAXIMUM 42 TABLETS. That is 14 days at three tablets daily. Supply only the number of days actually needed, up to that maximum. Do not round up to a pack.
Maximum treatment period	14 DAYS. No extension under this PGD. Total norethisterone for period delay must not exceed 30 days in any 6 month period, and no patient may be supplied more than twice in 6 months under this PGD.
Route and method	Oral.
Storage	Store below 25 degrees Celsius in a dry place, away from direct sunlight.
Drug interactions	Enzyme inducing medicines reduce efficacy and are handled as exclusions rather than cautions: rifampicin, rifabutin, carbamazepine, phenytoin, phenobarbital, primidone, topiramate, efavirenz, ritonavir and St John's wort. Also excluded: ciclosporin, and lamotrigine used as monotherapy. Check the current SPC and BNF against the patient's full medicine list, including anything bought over the counter or online.
Adverse effects	Common: nausea, headache, breast tenderness, mood changes, breakthrough bleeding, fluid retention. Rare but serious: venous thromboembolism, arterial thrombosis, jaundice and cholestatic liver injury, raised blood pressure, new migraine with aura. Refer to the current SPC.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	- That valid informed consent was given. - Patient name, address, date of birth, and the GP with whom they are registered. The date the last period started, and how pregnancy was excluded.

	• The reason the delay is wanted, and the dates it is needed for. • BMI, or the height and weight it was calculated from. • Smoking status, and the answer to the question about a journey of 4 hours or more. • Where the patient is 16 or 17: the competence assessment and the safeguarding consideration, in full, not just the conclusion. • Number of days supplied and number of tablets supplied. • Any previous supply for period delay in the last 6 months. • That the patient was told norethisterone is not a contraceptive, and told to test for pregnancy if her period does not arrive. • Name and registration number of the healthcare professional supplying. • Advice given, including advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults 18 and over: 8 years. Under 18s: until the 25th birthday, or the 26th where the patient was 17 when treatment finished.
Disposal	Advise the patient to return any unused tablets to a pharmacy.

Information for the patient

Written information	Supply the patient information leaflet provided with the product.
Counselling	• Start 3 days before your period is due. Take one tablet three times a day, every day, until you stop. • Your period will usually start 2 to 3 days after you take the last tablet. • THIS IS NOT CONTRACEPTION. You can get pregnant while taking it. Use condoms or another method. • If your period does not come within a few days of finishing, do a pregnancy test. • Move around and keep well hydrated, particularly on any journey where you are sitting for a long time. • Some nausea, headache, breast tenderness, mood change or spotting is common and settles when you stop.
Follow-up, and when to get help urgently	Stop the tablets and get medical help the same day if you develop any of these: • Pain, swelling, redness or warmth in one leg, particularly the calf. • Sudden chest pain, sudden breathlessness, or coughing up blood. • A severe or unusual headache, a new migraine, or any change in your vision or hearing. • Yellowing of the skin or the whites of the eyes. • Weakness or numbness down one side, or difficulty speaking. Call 999. For anything else, contact the pharmacy or your GP. If your period has not arrived within a week of finishing the course, do a pregnancy test and see your GP.

Appendix 1: The venous thromboembolism gate, as a checklist

Ask all eight. Any single YES excludes. Record the answers, not just the outcome.

1. Have you ever had a blood clot, a DVT, a clot on the lung, a stroke, a mini-stroke or a heart attack?	• YES: exclude and refer.
2. Has anyone in your immediate family had a blood clot before the age of 45, or been told they have a clotting disorder?	• YES: exclude and refer.
3. Do you smoke at all?	• YES: exclude and refer. • Any amount, any age.
4. Height and weight, for BMI.	• BMI 30 or above: exclude and refer. • Measure or ask. Do not estimate.
5. Will you be on a flight, coach, train or car journey of 4 hours or more during the course, or in the 2 weeks after it?	• YES: exclude and refer. • This will exclude many holiday requests. That is the intended effect. Give the alternatives in Appendix 2.
6. Have you had an operation under general anaesthetic in the last 6 weeks, or do you have one planned?	• YES: exclude and refer.
7. Are you going to be immobile for any reason, for example a leg in plaster or bed rest?	• YES: exclude and refer.
8. Do you have cancer now, or have you been treated for cancer in the last 12 months?	• YES: exclude and refer.

Appendix 2: What to offer a woman who is excluded

An exclusion is not the end of the conversation. Every excluded woman should leave with something.

- **If she takes a monophasic combined oral contraceptive: she can run packs back to back and skip the pill-free interval, for up to 3 consecutive packs. This is safer than norethisterone, needs no extra supply, and she can do it herself. Explain how.**
- If she takes an everyday combined preparation: skip the placebo tablets and start the active tablets of the next pack.
- If she takes a progestogen-only pill: this cannot reliably delay a period. Do not advise doubling up. Refer.
- If she has an implant or injection: these often cause irregular bleeding or no bleeding, and have no role in actively delaying a period.
- **If she is excluded on venous thromboembolism grounds: her GP can assess her individually and may still decide to prescribe. Say so, so that she does not hear a flat no.**
- Practical measures for the event itself: menstrual cups, period underwear, and tranexamic acid or an NSAID for heavy or painful bleeding, subject to the usual checks.

Appendix 3: Key references

- NICE Clinical Knowledge Summary: Contraception, delaying or stopping periods.
- Faculty of Sexual and Reproductive Healthcare guidance on combined hormonal contraception, for the extended-use option in Appendix 2.
- Summary of Product Characteristics for norethisterone 5mg tablets, current version.
- British National Formulary.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, <https://www.nice.org.uk/guidance/mpg2>

Authorisation

Version	v005
Supersedes	Version 004, 9 September 2026
Valid from date	9 September 2026
Expiry date	31 July 2027

Doctor	Name: Nitin Shori Job title: Medical Director, Get Real Health GMC: 6047293 Signed: N. Shori Date: 9 September 2026
Pharmacist	Name: Chris Pilkington Job title: Head Pharmacist, Get Real Health GPhC: 2046322 Signed: C. Pilkington Date: 9 September 2026

Both authorising signatories reviewed this version together on 9 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: Job title and organisation: Signature: Date:
Professional group signatory	Name: Job title and organisation: Signature: Date:

Change history

Version	v004
Date	8 September 2026
Changes	• The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. • The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. • Full clinical review and reissue. Re-read the document before working to it; the numbers have changed. • Maximum treatment period is 14 days. The previous document said up to 14 days in its summary and up to 17, extendable to 28, in its operative box. • Maximum quantity is 42 tablets, being 14 days at 5mg three times daily. The previous maximum was 90 tablets, which is 30 days. • Venous thromboembolism risk factors are now exclusions rather than cautions, set out as an eight question checklist in Appendix 1: personal or family history, current smoking of any amount at any age, BMI 30 or above, a seated journey of 4 hours or more during or within 2 weeks of the course, surgery within 6 weeks, immobility, and active or recently treated cancer. • A seated journey of 4 hours or more is an exclusion. This will exclude many holiday requests, which is the intended effect, and Appendix 2 sets out what to offer instead. • Age 16 and over is now formally authorised in a version signed by both signatories, with a required and recorded competence assessment, a safeguarding and coercion consideration, and a named referral route for every patient under 18. • Pregnancy exclusion made operable: the two questions to ask, when a pregnancy test is required, and the requirement to tell every patient to test if her period does not arrive. • Enzyme inducing medicines, ciclosporin and lamotrigine monotherapy moved from a general interactions statement into named exclusions. • Migraine with aura, uncontrolled hypertension or a blood pressure of 140/90 or above measured today, and diabetes with vascular complications added as exclusions. • Repeat supply limited to twice in any 6 months, and total treatment to 30 days in any 6 months. • Follow-up advice rewritten. The previous document carried template treatment-PGD wording telling patients to seek advice if symptoms "do not improve in 3-4 weeks", which is meaningless here. Replaced with named clot, liver and neurological symptoms requiring same-day help. • Alternatives for excluded women added as Appendix 2, including

	running monophasic combined pill packs back to back. ● Broken NICE reference corrected from mg2 to mpg2.
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Previous versions

001	Development and issue of new PGD. Norethisterone 5mg, women aged 18 and over. An amendment lowering the age criterion to 16 was recorded in the change history in July 2026 but was not reflected in any signed version; that change is properly authorised for the first time in v002.
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Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Premises name
 Address
 Postcode:
 ODS code, if applicable

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

[illegible]

RISK FACTORS THAT DO NOT EXCLUDE ON THEIR OWN

These are UKMEC 2025 category 2 for combined hormonal contraception: the advantages generally outweigh the risks, so ONE of them alone does NOT exclude. Supply, and counsel on the precautions below.

A flight or other seated journey of 4 HOURS OR MORE, during the course or within 2 weeks of finishing it.

CURRENT SMOKER AND UNDER 35, any number. UKMEC 2.

BMI 30 to 34.9 kg per square metre. UKMEC 2.

AGED 40 OR OVER. UKMEC 2.

TWO OR MORE PRESENT TOGETHER: EXCLUDE and refer. This is UKMEC 2025's own instruction: where multiple category 2 conditions relate to the same risk, clinical judgement must decide whether the risks outweigh the benefits. All four point at the same vascular risk, so they accumulate.

Record which risk factors were present and the count, not only the conclusion.

PRECAUTIONS TO GIVE WHERE A LONG JOURNEY IS INVOLVED: move around at least hourly, keep well hydrated, avoid alcohol and sedatives on the journey, and consider graduated compression stockings. Seek urgent help for a painful swollen calf, sudden breathlessness or chest pain, during the course or in the weeks after it.

A NOTE ON USING UKMEC FOR A NON-CONTRACEPTIVE INDICATION

UKMEC states in terms that its recommendations are for CONTRACEPTIVE purposes only, and that where a method is used for a non-contraceptive indication the risk and benefit profile and eligibility criteria may differ. Period delay is not contraception.

UKMEC is therefore used here BY ANALOGY, not as directly governing criteria. The analogy is justified by the Primolut N SPC, which states that norethisterone is partly metabolised to ethinylestradiol, giving roughly 4 to 6 micrograms of ethinylestradiol per 1 mg of norethisterone, and that because of this the general warnings associated with combined oral contraceptives should also be considered.

At 5 mg three times daily that conversion implies an ethinylestradiol equivalent above that of a standard 30 to 35 microgram combined pill. That calculation is derived from the SPC figures, not stated in the SPC itself, and is recorded as derived so that nobody cites it as a licensed statement.

The current edition is UKMEC 2025, published December 2025 by the College of Sexual and Reproductive Healthcare, which was renamed from the Faculty of Sexual and Reproductive Healthcare in August 2025. Earlier versions of this PGD would have cited UKMEC 2016, which is superseded.

BRAND MATTERS IN THIS PGD

The three norethisterone 5mg products licensed in the UK do NOT carry the same safety information, and the differences bear directly on this service.

PRIMOLUT N (Bayer) is the only one that states the metabolism to ethinylestradiol, the combined-oral-contraceptive warnings that follow from it, smoking and age as thromboembolic risk factors, arterial thromboembolic risk, a maximum duration of 10 to 14 days for postponement, and depression aggravated as an adverse reaction.

UTOVLAN (Pfizer) carries the depression warning in its special warnings but not the ethinylestradiol conversion or the duration limit.

NORETHISTERONE 5mg TABLETS (Wockhardt) carries neither the depression warning nor the ethinylestradiol conversion, and states no maximum duration.

WHICHEVER BRAND IS SUPPLIED, THE WARNINGS IN THIS PGD APPLY. They are drawn from the fullest of the three. Do not read a shorter leaflet as permission to do less.

Version and change record

Version: v005

Issued: 9 September 2026

Supersedes: the previous signed version of this document, and any correction notice attached to it.

Change: An appendix heading carried "added at version 004" and a paragraph headed "why this changed" explained what versions 002 and 003 had got wrong, in the body of the document. Both removed. The rationale is in the change history for version 004 and remains there. No clinical change.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Signed on behalf of Get Real Health

Version v005, 9 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		9 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		9 September 2026

Both authorising signatories reviewed this version together on 9 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.